

Prime Medicine

NASDAQ : PRME · \$570M mkt cap · 181M sh · \$149M cash, zero debt · ~\$149M cash = ~9-month runway; FY2025 10-K flags GOING-CONCERN doubt; Young-Company \$200M ATM; Pre-product prime editing (search-and-replace; David Liu / Broad lineage); burn ~\$43M/qtr; PM359 first-ever clinical prime edit (NEJM Dec'25); May'25 restructuring → in-vivo liver (Wilson/AATD)

\$3.16

THE CENTRAL QUESTION

PRME proved the first-in-human prime edit, but holds only ~9 months of cash against a going-concern flag and a \$200M ATM — does the platform tail survive the dilution before it pays off?

Prime Medicine is a clinical-stage prime-editing biotech — the precise 'search-and-replace' gene edit (no double-strand break), David Liu's sister technology to BEAM's base editing. Pre-revenue at \$3.16 (~\$585M cap), it proved the first-ever clinical prime edit (PM359, NEJM Dec 2025). But cash is ~\$149M (~9 months), the FY2025 10-K flags going-concern doubt, and a \$200M ATM means severe dilution at a ~\$3 stock; a May 2025 restructuring refocused it on in-vivo liver (Wilson's, AATD). The DCF asks whether the platform tail can outrun forced dilution. Weighted fair value \$3.39 vs \$3.16 — roughly fair, an option where the platform almost exactly offsets the balance sheet.

PROBABILITY-WEIGHTED EXPECTED VALUE

\$3.39

expected fair value today
+7.2% vs spot \$3.16

FORWARD COMPOUNDED VALUE

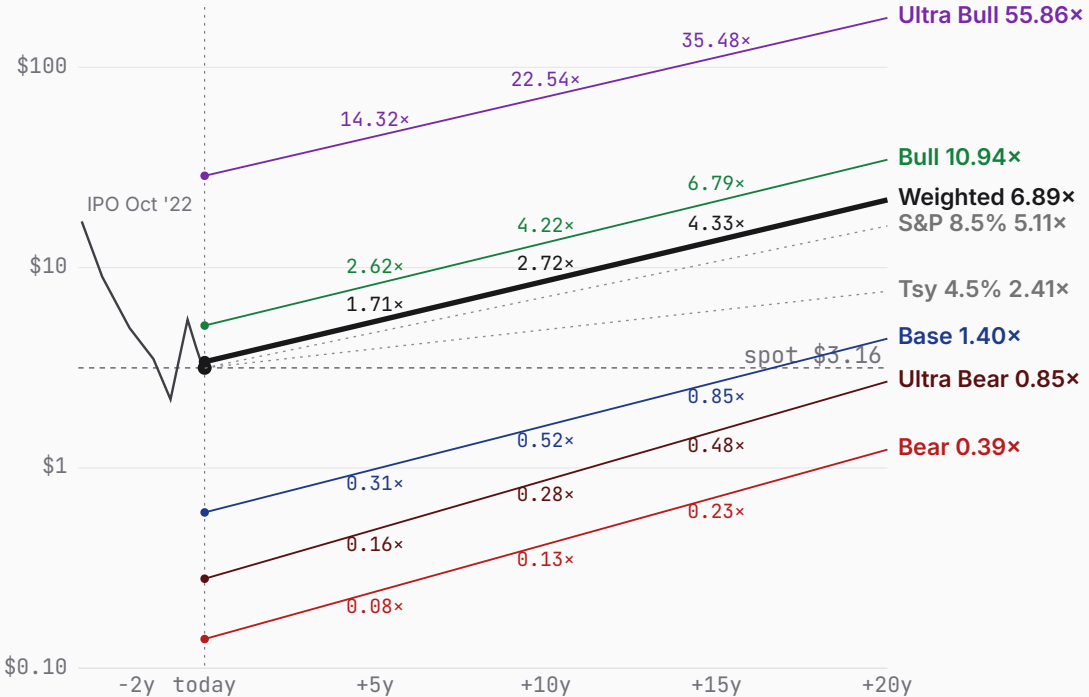
+5y	+10y	+15y	+20y
\$5.39	\$8.58	\$13.67	\$21.78
1.71× spot	2.72× spot	4.33× spot	6.89× spot

WEIGHTING RATIONALE

- Ultra Bear 16% Programs fail or cash runs out; ATM dilutes to the \$0.40 floor.
- Bear 30% One program limps; repeated raises at low prices overwhelm holders.
- Base 30% A liver program clears + BMS pays, but ~\$0.9B of raises nearly triples shares.
- Bull 16% Multiple liver successes; milder dilution as proof lifts the financing price.
- Ultra Bull 8% Prime editing wins broadly in-vivo; cleanest share count, platform terminal.

Bull (16%) + Ultra Bull (8%) together contribute \$3.12 — 92% of the \$3.39 expected value despite 24% combined probability. Today's spot (\$3.16) sits 7% below the weighted expected; forward-weighted value crosses spot between +5y and +10y.

PRICE + PROJECTIONS · HISTORICAL THROUGH PROBABILITY-WEIGHTED FORWARD



ULTRA BEAR	\$0.28	BEAR	\$0.14	BASE	\$0.60	BULL	\$5.14	ULTRA BULL	\$28.74
	-91.1% vs spot		-95.6% vs spot		-81.0% vs spot		+62.7% vs spot		+809.5% vs spot
TAM 0.4% P(fail) 90% S2C 1.0 Prob 16%		TAM 1.0% P(fail) 70% S2C 1.1 Prob 30%		TAM 1.7% P(fail) 55% S2C 1.2 Prob 30%		TAM 3.3% P(fail) 30% S2C 1.4 Prob 16%		TAM 6.7% P(fail) 18% S2C 1.6 Prob 8%	
Programs fail; dilutive death-march	One program limps; dilution wins		A liver program ships; dilution offsets		Liver successes; platform validates		Prime editing wins broadly in-vivo		

PROBABILITY WEIGHTS

Ultra Bear 16% Bear 30% Base 30% Bull 16% Ultra Bull 8% · Weighted expected \$3.39 (+7.2% vs spot)

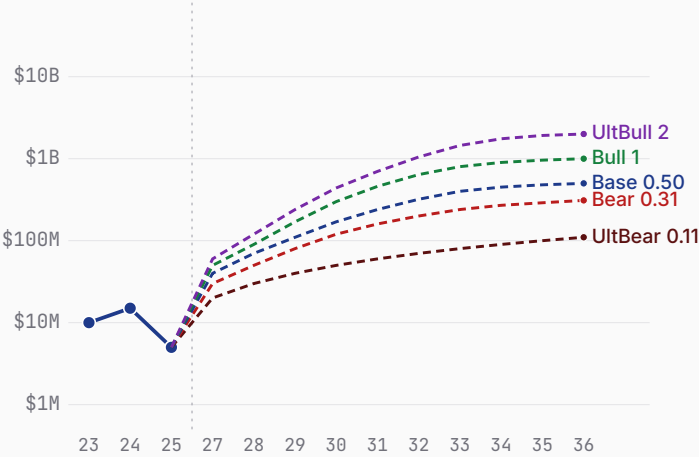
ULTRA BEAR	\$0.28	BEAR	\$0.14	BASE	\$0.60	BULL	\$5.14	ULTRA BULL	\$28.74
	-91.1% vs spot		-95.6% vs spot		-81.0% vs spot		+62.7% vs spot		+809.5% vs spot
Probability 16%		Probability 30%		Probability 30%		Probability 16%		Probability 8%	
Programs fail; dilutive death-march		One program limps; dilution wins		A liver program ships; dilution offsets		Liver successes; platform validates		Prime editing wins broadly in-vivo	
WHY 16%		WHY 30%		WHY 30%		WHY 16%		WHY 8%	
16% — the combination of 9-month runway, going-concern doubt, and clinical-stage risk makes near-term insolvency or a dilutive collapse a genuine, not tail, outcome.		30% — the most likely single path: the science shows enough to keep going but not enough to raise cheaply, so dilution erodes the per-share claim faster than value compounds.		30% — co-equal modal case: clinical proof exists (PM359) and the liver targets are credible, but the model auto-sizes the raises needed to fund them, and that dilution is the binding constraint on per-share value.		16% — requires both clinical breadth across liver targets and a financing path that improves as proof accumulates; plausible given the NEJM precedent, but it needs the science to de-risk before the cash forces bad raises.		8% — the platform-validation tail: requires prime editing to beat base editing and nuclease approaches on versatility and safety at scale, and the equity to survive intact long enough to capture it.	
WHAT HAPPENS		WHAT HAPPENS		WHAT HAPPENS		WHAT HAPPENS		WHAT HAPPENS	
Clinical programs miss and the cash runs out before any partner steps in. With ~9 months of runway and a going-concern flag, the \$200M ATM is tapped repeatedly into a falling stock, sinking the equity toward the \$0.40 distress floor.		A single program advances but slowly, forcing repeated capital raises through the ATM at depressed prices. The platform partly works, yet the financing dominates the equity story — every milestone is paid for in shares.		A Wilson's or AATD in-vivo program reaches market and BMS milestones (>\$3.5B potential) start landing, validating prime editing as a real modality. The business survives and generates revenue at ~37% drug margins.		Several in-vivo liver programs succeed and the platform partly validates as a versatile precise editor, drawing partners and pricing power. Revenue scales toward ~\$1.00B at 40% margins, and earlier validation means raises come at higher prices.		Prime editing establishes itself as a broad in-vivo platform across many edit types, the durable advantage David Liu's chemistry promises. Revenue reaches ~\$2.00B at 45% margins, and validation lets PRME fund growth without punishing dilution.	
FY35 product revenue stalls at ~\$0.11B and p_fail is 90% — survival itself is the bet, not just the science. The share count balloons from 180M to ~702M, so even residual value is spread thin. DCF ~\$0.81; the distress-weighted expected value is ~\$0.28.		FY35 revenue reaches ~\$0.31B with p_fail still 70%. The share count rises to ~568M, diluting surviving holders before economics arrive. DCF ~\$0.46; expected value ~\$0.14 — value is created, but new shares capture most of it.		But getting there costs ~\$0.9B of dilutive raises at depressed prices, nearly tripling the share count to ~500M. FY35 revenue ~\$0.50B; p_fail 55%. DCF \$0.84, expected ~\$0.60 — per-share value lands just below spot because dilution claims the upside.		Because the equity is less distressed, dilution is milder — the share count rises only to ~372M, not the 500M+ of the base. p_fail falls to 30%. DCF \$7.17, expected ~\$5.14 — the first scenario where the platform clearly outruns the balance sheet.		The share count rises only to ~285M — the cleanest balance-sheet outcome — and p_fail drops to 18%. DCF \$34.96, expected ~\$28.74. This is the platform-wins tail that the distressed entry price makes cheap to own.	

Prime Medicine business snapshot

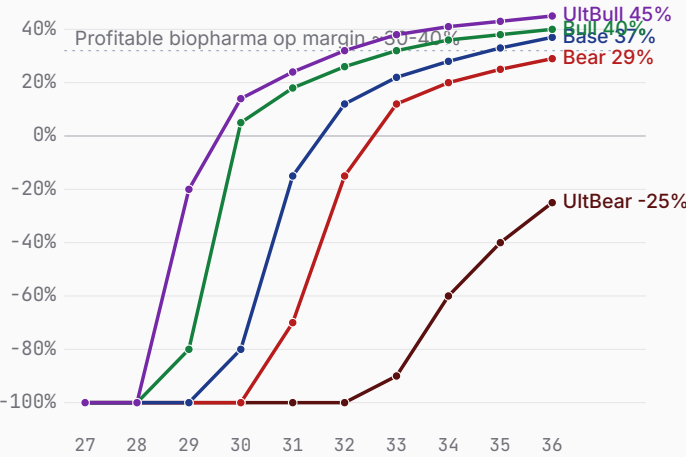
Bear \$0.14 Base \$0.60 Bull \$5.14

FY23-FY25 history + FY26-FY35 scenario projections · fiscal years end Dec 31 · Q1'26 (May 2026) release

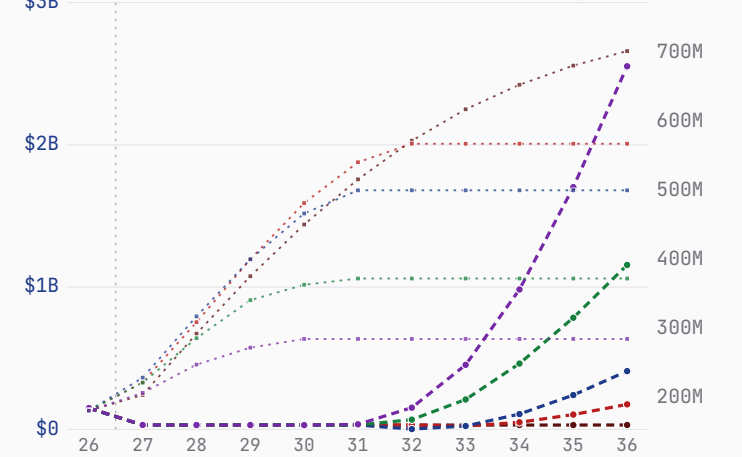
Revenue — history + scenarios to FY36 (log)



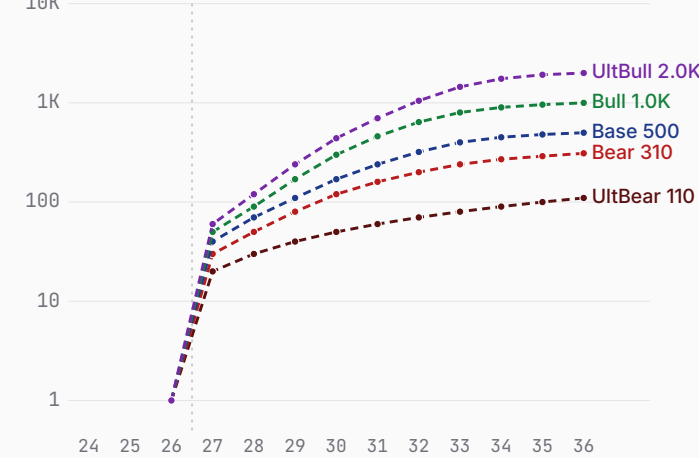
Path to profitability — op margin (FY27→FY36)



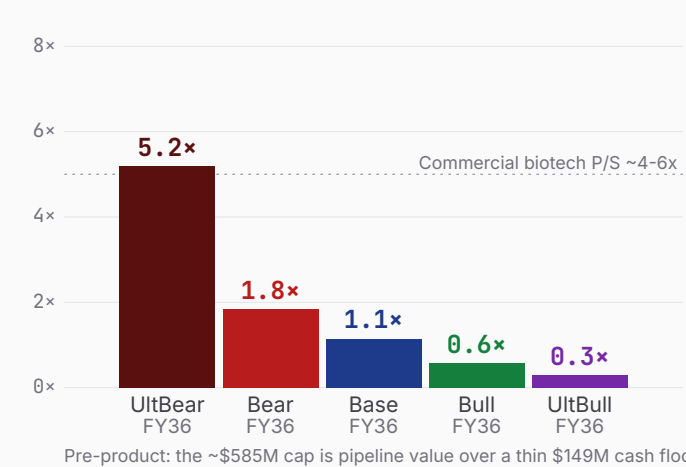
Cash + dilution (FY26 → FY36)



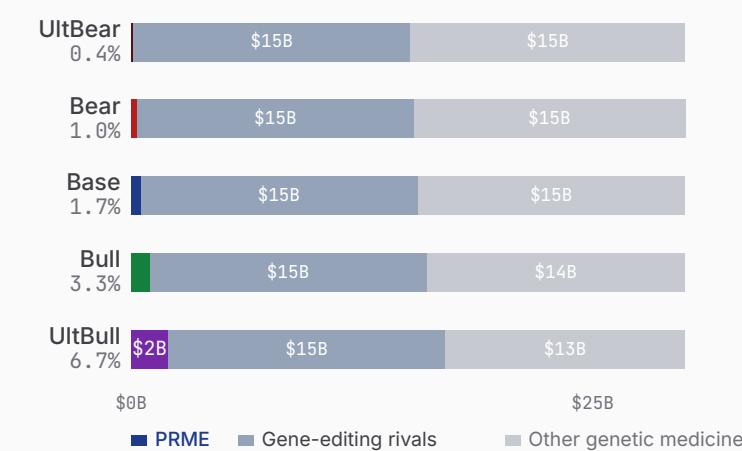
Revenue ramp (log)



\$0.57B mkt cap as P/S on FY36 rev



\$30B genetic-disease therapeutics — PRME share



Sources: PRME FY2025 10-K (going-concern) + Q1'26 results (CIK 1894562), PM359 CGD data (NEJM Dec 2025), May'25 restructuring, BMS/CFF agreements. Revenue is minimal collaboration income; product revenue modeled post-approval; raises auto-sized (the \$200M ATM dilution path). FCF is NOL-shielded pre-tax over the explicit window. TAM: a serviceable slice of genetic-disease therapeutics, ~\$30B.

Prime Medicine 7 Powers · the moat, scored

Bear \$0.14 Base \$0.60 Bull \$5.14

Arena. In-vivo and ex-vivo precise gene editing for genetic disease, against base editors (BEAM), nuclease editors (Intellia, CRISPR Therapeutics) and editing platforms (Editas).

POWER ORIENTATION
origination window: closing

POWER ORIENTATION · 7 POWERS

Scale Economies	Sub-scale and cash-starved; forced to cut ~25% of staff and the pipeline in May 2025.
Network Economies	No network effects in a therapeutics platform; value is clinical proof, not adoption density.
Counter-Positioning	Prime editing avoids double-strand breaks and makes edit types base editing cannot — a model rivals can't simply copy.
Switching Costs	Switching costs accrue only post-approval, per indication; none yet with zero approved products.
Branding	No commercial brand; pre-revenue clinical-stage with reputational, not pricing, recognition.
Cornered Resource · thesis leans here	David Liu / Broad prime-editing IP and know-how are a genuine scarce asset — but underfunded to exploit.
Process Power	Editing-design and delivery know-how is real but early and not yet a durable manufacturing advantage.

COMPETITIVE READ

PRME is trying to originate a cornered-resource/process Power in versatile, precise editing — the David Liu prime-editing platform is a genuine scarce asset, and PM359 proves the chemistry works in humans. But the company is cash-starved and was forced to cut its pipeline, so the Power-origination window is closing faster than the science is maturing. The falsifier the bull must clear is concrete — reach a partnered or approved program before the balance sheet forces value-destroying dilution. Same family as BEAM, opposite balance sheet.

THE RACE · NAMED RIVALS

BEAM (base editing)	\$1.21B cash, runway to ~2029
~30% terminal share · Same gene-editing family; \$1.21B cash vs PRME's \$149M — far better capitalized.	
Intellia / NTLA	well-funded, partnered
~25% terminal share · In-vivo CRISPR nuclease editing; deeper clinical pipeline and balance sheet.	
CRISPR Therapeutics	product revenue + large cash
~20% terminal share · Approved ex-vivo product (Casgevy) and substantial cash; nuclease leader.	
Editas Medicine	constrained
~5% terminal share · Editing platform peer; also capital-constrained and restructuring.	

LEAD / LAG · FALSIFIERS

Clinical first	PM359 first-ever clinical prime edit (NEJM) vs Base/nuclease peers further in pivotal trials	EVEN
Balance sheet	~\$149M / ~9 months / going-concern vs BEAM ~\$1.21B / runway to ~2029	LAGGING
In-vivo delivery	Refocused to liver, pre-clinical-readout vs NTLA/peers with in-vivo human data	LAGGING

Prime Medicine show your work

Bear \$0.14 Base \$0.60 Bull \$5.14 · Weighted \$3.39 (+7.2%)

ULTRA BEAR

\$0.28

BEAR

\$0.14

BASE

\$0.60

BULL

\$5.14

ULTRA BULL

\$28.74

ASSUMPTIONS

TAM Year-10 (\$B)	30
Mature share (%)	0.4
Mature op margin (%)	-25
Sales-to-capital	1.0
Terminal WACC (%)	12.0
Terminal growth (%)	3.0
P(failure) (%)	90
Scenario probability (%)	16

10-YEAR DCF · \$B

FY	Rev	Margin	FCF	PV FCF
FY27	0.02	-700%	-0.14	-0.12
FY28	0.03	-500%	-0.16	-0.12
FY29	0.04	-350%	-0.15	-0.10
FY30	0.05	-250%	-0.14	-0.08
FY31	0.06	-180%	-0.12	-0.06
FY32	0.07	-130%	-0.10	-0.04
FY33	0.08	-90%	-0.08	-0.03
FY34	0.09	-60%	-0.06	-0.02
FY35	0.10	-40%	-0.05	-0.01
FY36	0.11	-25%	-0.04	-0.01

Σ PV explicit FCF	-0.60
+ PV terminal (g 3.0%)	-0.12

EQUITY BUILD · \$B & PER SHARE

Operating EV	\$-0.72B
+ Cash & investments	\$0.15B
= Equity value	\$-0.57B
Raised over period	\$0.94B
Dilution by FY36	+289%
FY36 shares (M)	702.3
DCF per share	\$-0.81
× (1-P_fail) [10%]	\$-0.08
+ P_fail × distress [90% × \$0.40]	\$0.36

= Expected per share \$0.28

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
\$0.49	\$0.87	\$1.53	\$2.70
0.16×	0.28×	0.48×	0.85×

ASSUMPTIONS

TAM Year-10 (\$B)	30
Mature share (%)	1.0
Mature op margin (%)	29
Sales-to-capital	1.1
Terminal WACC (%)	11.5
Terminal growth (%)	3.5
P(failure) (%)	70
Scenario probability (%)	30

10-YEAR DCF · \$B

FY	Rev	Margin	FCF	PV FCF
FY27	0.03	-650%	-0.20	-0.18
FY28	0.05	-400%	-0.22	-0.17
FY29	0.08	-250%	-0.23	-0.15
FY30	0.12	-140%	-0.20	-0.12
FY31	0.16	-70%	-0.15	-0.08
FY32	0.20	-15%	-0.07	-0.03
FY33	0.24	+12%	-0.01	-0.00
FY34	0.27	+20%	+0.03	+0.01
FY35	0.29	+25%	+0.05	+0.02
FY36	0.31	+29%	+0.07	+0.02

Σ PV explicit FCF	-0.68
+ PV terminal (g 3.5%)	0.27

EQUITY BUILD · \$B & PER SHARE

Operating EV	\$-0.41B
+ Cash & investments	\$0.15B
= Equity value	\$-0.26B
Raised over period	\$0.97B
Dilution by FY36	+214%
FY36 shares (M)	567.8
DCF per share	\$-0.46
× (1-P_fail) [30%]	\$-0.14
+ P_fail × distress [70% × \$0.40]	\$0.28

= Expected per share \$0.14

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
\$0.24	\$0.42	\$0.72	\$1.23
0.08×	0.13×	0.23×	0.39×

ASSUMPTIONS

TAM Year-10 (\$B)	30
Mature share (%)	1.7
Mature op margin (%)	37
Sales-to-capital	1.2
Terminal WACC (%)	10.5
Terminal growth (%)	4.0
P(failure) (%)	55
Scenario probability (%)	30

10-YEAR DCF · \$B

FY	Rev	Margin	FCF	PV FCF
FY27	0.04	-550%	-0.24	-0.21
FY28	0.07	-320%	-0.25	-0.19
FY29	0.11	-180%	-0.23	-0.16
FY30	0.17	-80%	-0.19	-0.11
FY31	0.24	-15%	-0.09	-0.05
FY32	0.32	+12%	-0.03	-0.01
FY33	0.40	+22%	+0.02	+0.01
FY34	0.45	+28%	+0.08	+0.03
FY35	0.48	+33%	+0.13	+0.05
FY36	0.50	+37%	+0.17	+0.05

Σ PV explicit FCF	-0.60
+ PV terminal (g 4.0%)	0.87

EQUITY BUILD · \$B & PER SHARE

Operating EV	\$0.27B
+ Cash & investments	\$0.15B
= Equity value	\$0.42B
Raised over period	\$0.90B
Dilution by FY36	+177%
FY36 shares (M)	500.2
DCF per share	\$0.84
× (1-P_fail) [45%]	\$0.38
+ P_fail × distress [55% × \$0.40]	\$0.22

= Expected per share \$0.60

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
\$0.99	\$1.63	\$2.68	\$4.42
0.31×	0.52×	0.85×	1.40×

ASSUMPTIONS

TAM Year-10 (\$B)	30
Mature share (%)	3.3
Mature op margin (%)	40
Sales-to-capital	1.4
Terminal WACC (%)	10.0
Terminal growth (%)	5.0
P(failure) (%)	30
Scenario probability (%)	16

10-YEAR DCF · \$B

FY	Rev	Margin	FCF	PV FCF
FY27	0.05	-450%	-0.25	-0.22
FY28	0.09	-220%	-0.23	-0.18
FY29	0.17	-80%	-0.19	-0.14
FY30	0.30	+5%	-0.08	-0.05
FY31	0.46	+18%	-0.03	-0.02
FY32	0.64	+26%	+0.04	+0.02
FY33	0.80	+32%	+0.14	+0.07
FY34	0.90	+36%	+0.25	+0.11
FY35	0.96	+38%	+0.32	+0.12
FY36	1.00	+40%	+0.37	+0.13

Σ PV explicit FCF	-0.16
+ PV terminal (g 5.0%)	2.68

EQUITY BUILD · \$B & PER SHARE

Operating EV	\$2.52B
+ Cash & investments	\$0.15B
= Equity value	\$2.67B
Raised over period	\$0.67B
Dilution by FY36	+106%
FY36 shares (M)	372.3
DCF per share	\$7.17
× (1-P_fail) [70%]	\$5.02
+ P_fail × distress [30% × \$0.40]	\$0.12

= Expected per share \$5.14

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
\$8.28	\$13.33	\$21.47	\$34.58
2.62×	4.22×	6.79×	10.94×

ASSUMPTIONS

TAM Year-10 (\$B)	30
Mature share (%)	6.7
Mature op margin (%)	45
Sales-to-capital	1.6
Terminal WACC (%)	9.5
Terminal growth (%)	6.0
P(failure) (%)	18
Scenario probability (%)	8

10-YEAR DCF · \$B

FY	Rev	Margin	FCF	PV FCF
FY27	0.06	-350%	-0.23	-0.21
FY28	0.12	-140%	-0.20	-0.16
FY29	0.24	-20%	-0.12	-0.09
FY30	0.44	+14%	-0.06	-0.04
FY31	0.70	+24%	+0.01	+0.00
FY32	1.05	+32%	+0.12	+0.06
FY33	1.45	+38%	+0.30	+0.14
FY34	1.75	+41%	+0.53	+0.23
FY35	1.92	+43%	+0.72	+0.28
FY36	2.00	+45%	+0.85	+0.31

Σ PV explicit FCF	+0.53
+ PV terminal (g 6.0%)	9.28

EQUITY BUILD · \$B & PER SHARE

Operating EV	\$9.81B
+ Cash & investments	\$0.15B
= Equity value	\$9.96B
Raised over period	\$0.52B
Dilution by FY36	+58%
FY36 shares (M)	284.6
DCF per share	\$35.00
× (1-P_fail) [82%]	\$28.70
+ P_fail × distress [18% × \$0.40]	\$0.07

= Expected per share \$28.74

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
\$45.24	\$71.22	\$112.12	\$176.51
14.32×	22.54×	35.48×	55.86×

Prime Medicine

supporting analysis · disclaimers · glossary

Bear \$0.14 Base \$0.60 Bull \$5.14

PUSHBACK · WHY THE BASE CASE IS TOO HARSH

- 1

First-in-class proof is real
PM359 restored NADPH-oxidase activity in the first-ever clinical prime edit (NEJM Dec 2025) — the core chemistry works in humans, not just in vitro.
- 2

Liver refocus de-risks delivery
The May 2025 pivot to in-vivo liver (Wilson's, AATD) targets the best-validated delivery route, where LNP-to-liver is a solved problem for rivals.
- 3

Non-dilutive capital exists
BMS (\$55M upfront, \$55M equity, >\$3.5B milestones) and CFF (up to \$39M) offset some burn — not every dollar of progress must come from the ATM.
- 4

The de-rating prices the distress
At \$3.16, down from a \$6.94 high, much of the going-concern risk is already in the stock; the platform tail is being bought cheaply.
- 5

David Liu lineage
The Broad-Institute prime-editing IP and scientific lead are a genuine cornered resource that a partner or acquirer could fund through the cash crunch.

FALSIFICATION TRIGGERS

Bull validation A partnered or approved liver program (Wilson's/AATD), a PM359 partner/BLA path, or a financing/partnership that closes the runway gap before the ATM is leaned on.	Bear validation Repeated ATM draws at sub-\$3 prices, a clinical setback in the lead liver programs, or runway slipping below two quarters without a partner.	Reframe needed Insolvency, a recap, or a take-under would move this from 'distressed option' to 'broken' — re-underwrite from the floor, not the platform tail.
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GLOSSARY · CONCEPTS REFERENCED IN THE NARRATIVE

prime editing — Precise 'search-and-replace' gene edit that rewrites DNA without a double-strand break; makes many edit types. David Liu / Broad lineage.	PM359 / CGD — PRME's program for chronic granulomatous disease; the first-ever clinical prime edit (NEJM Dec 2025). Deprioritized; now seeking a partner/BLA path.	in-vivo liver editing — Editing genes directly inside the body in the liver — the focus after the May 2025 refocus (Wilson's disease PM577, AATD PM647).
going-concern — Auditor/management doubt, flagged in the FY2025 10-K, that the company has enough cash to operate for the next 12 months.	ATM / forced dilution — An 'at-the-market' equity program (\$200M here) that sells new shares into the open market; at a ~\$3 stock it heavily dilutes existing holders.	p_fail — Modeled probability the scenario ends in failure or insolvency before economics arrive — elevated here because surviving the cash crunch is itself uncertain.